

Wells Score — Pulmonary Embolism

adults, non-pregnant; not validated in pregnancy · casualty / ward

Wells PE

Criterion	Wells	Simplified Wells
Clinical signs/symptoms of DVT (min. leg swelling + pain on palpation of deep veins)	+3	+1
Alternative diagnosis less likely than PE	+3	+1
Heart rate >100 bpm	+1.5	+1
Immobilisation >3 days or surgery in previous 4 weeks	+1.5	+1
Previous DVT/PE	+1.5	+1
Haemoptysis	+1	+1
Malignancy (on treatment, treated in last 6 months, or palliative)	+1	+1
Total (range)	0 to 12.5	0 to 7

INTERPRETATION

Wells · three-tier	Low (<2) PE ≈1.3%	Moderate (2-6) PE ≈16%	High (>6) PE ≈38%
Wells · two-level (modified)	PE unlikely (≤4)	PE likely (>4)	
Simplified Wells · two-level	PE unlikely (0-1)	PE likely (≥2)	

PEARLS

- ▶ Apply only after a history and examination suggest PE
- ▶ Haemodynamically unstable → not a Wells problem: resuscitate, bedside echo, treat
- ▶ Do not D-dimer 'likely' patients — go straight to imaging
- ▶ PERC not validated in COVID-19

NEXT STEPS

PE likely (>4)

- ▶ CTPA immediately if possible
- ▶ If CTPA delayed → interim therapeutic anticoagulation
- ▶ CTPA negative + DVT suspected → proximal leg vein ultrasound

PE unlikely (≤4)

- ▶ D-dimer (result within 4 h); anticoagulate if delayed
- ▶ D-dimer positive → CTPA (as for 'likely')
- ▶ D-dimer negative → PE excluded; stop anticoagulation

PERC RULE

Low suspicion (gestalt <15%): PE excluded if NONE of the 8 present

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| ☐ Age ≥50 | ☐ HR ≥100 |
| ☐ SaO ₂ <95% | ☐ Haemoptysis |
| ☐ Oestrogen use | ☐ Surgery/trauma ≤4 weeks |
| ☐ Previous DVT/PE | ☐ Signs of DVT |

Age-adjusted D-dimer cut-off (age >50): cut-off (µg/L FEU) =
age × 10 · use if assay cut-off is 500 µg/L FEU

SA GUIDELINES

- Diagnosis is clinical, confirmed with imaging +
- Unstable confirmed PE: thrombolysis — discuss with specialist +
- Rx: rivaroxaban 15 mg 12-hourly × 3 wk, then 20 mg daily × 3 months (check contraindications, Table 2.5) +
- Pregnancy: strong suspicion → enoxaparin 1 mg/kg SC 12-hourly + refer to confirm +